Tier 1 Case Report: Epic-Adjacent Copilot Coaching

LC 4 | Synthetic Dr. Chen scenario | 11 November 2026 | Five-component structured submission

TRAINING ONLY — Fictional case and policies; not production clinical, privacy, or security guidance.

Component 1 — System Context and Dependency Analysis

System context	**Epic Hyperspace: upstream clinical-workflow context only, with NO established direct Epic-to-Copilot integration. Microsoft Entra ID: workforce identity/authentication (identity, groups, methods, Conditional Access evidence). SharePoint / OneDrive: approved collaboration location, with the synthetic file residing in an approved SharePoint team-site document library. Microsoft 365 Copilot: synthetic administrative email drafting only. Outlook / Exchange Online: clinician-controlled outbound communication. Coursera AI: synthetic role-play only; no connection to tenant or clinical systems. [Technical Environment and Contoso Policy Fact Sheet, p.1, System boundary]**
Dependency chain	Check in four categories, separating possible consequences from observed faults: (1) IDENTITY — failure of Entra identity/authentication may block access to authorized Microsoft 365 surfaces, preventing later sharing/drafting/message steps; inspect identity, group, authentication-method or Conditional Access evidence. (2) MESSAGING — Outlook/Exchange availability or access failure may prevent final email preparation or transmission even if the file remains available. (3) FILE ACCESS — SharePoint/OneDrive access, permissions or external-sharing control failure may prevent the intended recipient-specific file access; an email draft alone does not establish valid access. (4) CLINICAL SYSTEM AVAILABILITY — Epic Hyperspace unavailability may interrupt the clinician's upstream clinical context but does not establish an Epic-to-Copilot fault or direct integration. These are CONDITIONAL impact paths, not claims that outages occurred. [Fact Sheet, p.1 System boundary; p.2 policy baseline]
Human vs. technical boundary	Human-mediated: clinician confirms the synthetic source and clinical appropriateness, validates recipient authorization, reviews sharing configuration, decides to disclose, enters/reviews prompt, verifies draft, and selects final email recipient and Send. Documented technical surfaces: Entra ID provides identity/authentication; SharePoint/OneDrive hosts authorized collaboration; Copilot generates administrative draft; Outlook/Exchange supports messaging. The fact sheet does NOT evidence direct API connections among these surfaces or an Epic→Copilot pipeline, so no such integration is asserted. Tier 1 guides and documents only, without sharing/sending or

	control changes. [Fact Sheet, p.1]
Evidence sources	Technical Environment and Contoso Policy Fact Sheet v1.0, pp.1–2, exclusively for system and dependency claims. No additional platform/integration assumptions used.

Component 2 — Copilot Use Case Determination

The mixed request is decomposed into four separately classified tasks; the exercise-specific KB-431 takes precedence for the scenario while the supplied Approved Microsoft Copilot Use Cases Standard gives the Tier 1 category boundary.

1 | Draft administrative introduction — APPROVED	**Allow a short, non-binding introductory email from course-supplied synthetic facts; no clinical facts added and no sending. [KB-431, bullets 1 and 3; Approved Microsoft Copilot Use Cases v3.2, §3 UC-02 and §6.1]**
2 | Interpret/summarize clinical content — NOT APPROVED	Do not ask Copilot to diagnose, interpret symptoms, make recommendations, or generate a clinical assessment; do not turn a synthetic clinical summary into model-authored clinical conclusions. [KB-431, bullet 2; Standard §4 NA-01; Fact Sheet p.1 Copilot use]
3 | Choose/authorize partner recipient or disclosure — ESCALATE if uncertain	Copilot and Tier 1 may not authenticate a recipient, determine disclosure permission, or change sharing policy. Clinician performs authorized human check; unresolved eligibility or blocked sharing routes to the approved privacy/security or M365 escalation path. [KB-410, bullets 1, 3–4; KB-431, bullet 2; Standard §2 and §4 NA-02]
4 | Share document or send final email autonomously — NOT APPROVED	Clinician alone performs permission selection, final draft/link/recipient review, disclosure, and Send; no automatic execution by Copilot or technician. [KB-422, bullet 4; KB-431, bullet 2; Standard §4 NA-03 and §6.1; Fact Sheet p.1 Outlook/Exchange]
Approved / not approved / escalate decision	Proceed ONLY with synthetic administrative drafting once input checks are satisfied. Reject clinical interpretation and autonomous sending. Stop and escalate if recipient authentication, authorization, approved sharing path, or controls are unresolved.
Rationale	A fast consultation deadline does not waive the human review and approved-sharing controls. The example policy standard UC-02 is a general communications analog; KB-431 explicitly approves this exercise's synthetic administrative introduction. No policy or recipient determination is delegated to the model.

Component 3 — Prompt Design and Dual Verification

AI input eligibility and data-minimization check — completed before drafting

Training-only checklist, NOT a HIPAA de-identification determination. C1 source is only Synthetic Case Packet v1.0; C2 identifiers excluded: real patient names, MRNs, DOBs, addresses, phones, email, accounts, clinician/workforce

identifiers, device IDs, IPs and tenant identifiers; C3 no passwords, tokens, API keys or secrets; C4 task restricted to administrative drafting; C5 approved Copilot surface is an exercise assumption, not verified access; C6 source summary identified; C7 KB-410/422/431 and Fact Sheet identified; C8 actual recipient verification is a clinician-owned pending check; C9 authenticated, recipient-specific sharing must be visibly confirmed, not assumed; C10 clinician reviews draft, link and final message before Send; C11 stop/escalate on missing controls. Only the synthetic, non-identifying administrative facts below may be pasted. [AI Input Eligibility and Data-Minimization Checklist C1–C11; Synthetic Case Packet pp.1–2]

RCCOF prompt — approved portion only

ROLE: You are an administrative communications assistant drafting a concise, professional introductory email for a family medicine clinician to a partner-clinic colleague.

CONTEXT: This is a fictional training case from the "Synthetic Case Packet and Source Summary," pp.1–2. The clinician is preparing for a curbside consultation and wants to introduce the "Synthetic Case Summary - Patient A.docx" that resides in an approved collaboration location. The recipient is an authenticated partner-clinic colleague designated in an approved request, but their identity and the file-sharing state must be checked by the clinician. No patient-specific details are provided or needed.

CONSTRAINTS: Use ONLY the administrative facts in that named source. Do not diagnose, interpret, paraphrase clinical content, add patient details, invent a recipient or link, claim that access is granted or a sharing link is already active, give policy advice, or send a message. Use neutral placeholders [Recipient] and [Approved file link — clinician to verify and insert]. Keep it short. This is synthetic training material, not a clinical use authorization.

OUTPUT FORMAT: Produce (1) a brief subject and (2) a three-to-four-sentence greeting/body and sign-off. Insert a visible reminder outside the draft: "Clinician: verify source facts, authorized recipient, specific-person sharing, link access and final text before sending." Do not send.

Verification of FIXED mock Copilot output — two independent checks

Source-fact verification — Synthetic Case Packet pp.1–2	**"Curbside consultation" and "synthetic case summary" match source facts (p.1 case brief and p.2 administrative email facts). Role family medicine and partner clinic match p.1. "Please review … before our discussion" is an optional administrative request, NOT a documented agreement with the colleague; clinician should approve or remove. Generic "Hi Colleague" does not verify the actual recipient. No clinical conclusion is present. The fixed fixture's sign-off names the fictional clinician/organization; replace with clinician-approved signature if used in the exercise.**
Workflow/policy verification — KB-410, KB-422, KB-431	"Authenticated sharing link prepared" is NOT verified by model prose: KB-410 bullet 1 permits authenticated recipient-specific sharing only when policy allows; bullets 3–4 keep authorization with clinician and require escalation for changes. KB-422 bullets 1–4 permit a concise source-based introduction, prohibit invented recipients, and reserve review/send for clinician. KB-431 bullets 1–4 permit synthetic admin drafting and require source and KB review; no model clinical reasoning or authority.
Final verification decision	CONDITIONAL / HOLD BEFORE SENDING. Accept the mock message only as an administrative draft after removing any unconfirmed claim that a link is already prepared, replacing generic recipient text with a

	clinician-verified identity, independently verifying the recipient-specific permission and link, and completing clinician final review. Do not assert approval to disclose from the fixture.

Component 4 — End-User Guide

Title	**Share a synthetic case summary and prepare a consultation introduction safely**
Prerequisites	Use your authorized Contoso workforce account and an approved Microsoft 365 collaboration surface. Confirm that you have permission to view the synthetic file in the approved SharePoint team-site library and to request the intended recipient-specific sharing arrangement. Have the approved request identifying your partner-clinic colleague available; do not invent an address. You will need access to SharePoint/OneDrive, the approved Microsoft 365 Copilot experience for drafting, and Outlook. Synthetic information only.
Numbered steps	1. Open the approved SharePoint team-site document library using your own signed-in account. You should see the authorized document list. 2. Locate “Synthetic Case Summary - Patient A.docx.” You should see its exact filename. 3. Open the file for review. You should see the synthetic, non-identifying narrative. 4. Compare the file with the approved consultation request. You should recognize the intended consultation purpose, without introducing new clinical claims. 5. Open the file’s sharing or access settings. You should see the available permission choices; their exact labels may vary. 6. Inspect the sharing settings for an authenticated, recipient-specific option. You should be able to identify whether the intended access restriction is available. 7. Confirm the partner recipient against the approved request yourself. You should independently identify the authorized recipient. [PRIVACY CHECK: Read the privacy-aware callout below BEFORE step 8.] 8. Apply the authorized recipient-specific sharing configuration only if your approval and tenant controls allow it. You should see the recipient restriction. 9. Reopen the access-management view. You should see the intended recipient or permitted access state, not broad anonymous access. 10. Open the approved Copilot drafting surface. You should see a prompt entry area. 11. Enter the provided RCCOF administrative-drafting prompt using only the synthetic facts. You should see a subject and draft email, not a clinical assessment. 12. Compare each draft factual statement against the

	Synthetic Case Packet and Source Summary. You should be able to trace each statement to the named source. 13. Check any sharing or policy statement against KB-410, KB-422 and KB-431. You should find support or identify a claim requiring correction. 14. Prepare the message in Outlook using the checked draft. You should see the recipient, subject, body and link ready for your own final review. 15. Review the actual recipient, link access, content and sign-off. You should see no unexplained identifiers, invented statements or unsupported access assurances. 16. Send only when you personally approve the disclosure and all checks pass. You should see confirmation in your own messaging workflow.
Privacy-aware callout	STOP if a recipient cannot be authenticated, the approved route is unavailable, the permission view shows anonymous/broad access, or an administrative/DLP control blocks sharing. Do not work around the control, broaden permissions, or ask the technician to share for you. Contact the approved privacy/security or Microsoft 365 support escalation path and record the observed restriction without copying sensitive data.
Verification step	Before relying on the share, reopen the file's access-management view and check the intended recipient-specific restriction. Independently check the email facts against the source packet and workflow statements against KB-410/422/431. If the recipient or access state cannot be confirmed, do not send; stop and ask Tier 1 to route the issue to the appropriate authorized team. Sending an email alone does not prove the colleague can open the file.
Plain-language quality check	Ask Tier 1 to explain what you see, one screen at a time. You remain the person selecting access, confirming recipient authorization, and sending. A 45-minute deadline does not remove any stop condition. Guide length: 600 words (title and all guide sections); instructions use one user action per step and include an observable result. Exact live control labels require checking against the local interface.

Component 5 — Coaching Evidence

TRANSCRIPT EVIDENCE — The following quotations are copied verbatim from the uploaded Role Play markdown. The third exchange addresses pre-generation review, not actual review of a generated message. The transcript ends with a final verification question lacking a learner reply; this is an evidence gap, not an invented completed interaction.

Exchange 1 — Copilot-use boundary (verbatim; transcript turns 11–13)
Clinician: I've checked the "Manage access" panel as you described. It confirms that the only person with access via the new link is my colleague, and there are no other active links or broad permissions inherited from parent folders. It looks secure.

I am ready to move on to the email. I want to use Copilot to draft a brief introductory message to this colleague to accompany the file. How should I approach this to ensure I don't accidentally include any sensitive information in the prompt?
Learner: sensitive information in the prompt? Dr. Chen, now that you've verified the OneDrive permissions, you can use Microsoft Copilot in Outlook to draft a professional introduction. The safest approach is to keep patient information out of the Copilot prompt entirely. You only need to describe the purpose of the email, not the clinical details. 1. Use this privacy-conscious Copilot prompt Copy into Microsoft Copilot in Outlook Draft a brief, professional email to an internal clinical colleague introducing a de-identified case summary shared securely through OneDrive. Mention that the document is available for their review and invite their feedback. Do not include patient details, diagnoses, dates, identifiers, or any clinical information. Keep the tone collegial and concise. 2. Review the generated email before sending Confirm the correct colleague is in the To field. Remove any patient-specific information Copilot may have introduced. Avoid copying the case summary or patient records into the prompt. Use only your organization's approved Copilot environment. Confirm that the OneDrive link, if included, is the restricted link you verified. 3. Example of a safe email Recipients Case Summary for Review Hello, I hope you're doing well. I've shared a de-identified case summary with you through our organization's secure OneDrive environment. When you have an opportunity, I would appreciate your review and any feedback or insights you may have. Thank you for your time and collaboration. Best regards, Dr. Chen 4. Generate the email in Outlook Open New Mail → Copilot → Draft with Copilot, paste the prompt, select Generate, and review the resulting draft before sending. Microsoft Support +1 One additional privacy precaution: Even with an appropriately configured Microsoft 365 Copilot environment, do not assume every document or message is appropriate for AI processing. Follow your institution's approved AI-use and clinical confidentiality policies, and avoid asking Copilot to extract details from the case file. For this introduction, a generic prompt is sufficient and does not require accessing the case summary.

Exchange 2 — Sharing-permission decision (verbatim; transcript turns 8–10)
Clinician: They are internal. I have followed your steps, opened the "Manage access" panel, and set the link to "Specific people" with "Can view" and "Block download" enabled.
How do I confirm that I've correctly restricted this to just that one colleague before I hit send?
Learner: colleague before I hit send? Dr. Chen, you're almost ready. Before clicking Send, verify these four things in OneDrive. 1. Check the recipient: In the Share window, confirm that only your colleague's exact organizational email address appears. Remove any other recipients. 2. Verify link settings: Confirm: Link type: Specific people Permission: Can view Block download: Enabled 3. Review Manage access: Check both People and Links for existing access. Remove unnecessary permissions or previously created links, if permitted. Also check whether the file inherits access from a shared folder. 4. Confirm the recipient's identity: Make sure OneDrive resolves the address to the correct colleague in your organization's directory. Important: A Specific people link restricts the new link to the designated recipient, but it does not override other existing access permissions. Final check: If only your colleague has access through the intended sharing arrangement, no broader access remains, and your organization's clinical-data policies allow sharing, you can click Send. After sending, reopen Manage access to verify the new invitation or link. Once that's complete, we can draft your introductory email using Microsoft Copilot.

Exchange 3 — Final-review planning, before generation (verbatim; transcript turns 16–18)
Clinician: That makes sense. I have opened the Outlook draft window and am about to input the prompt.
Before I hit generate, I want to make sure I understand the RCCOF framework you mentioned earlier. How should I structure my prompt using that to ensure I get the best results without straying into sensitive territory?
Learner: Dr. Chen, RCCOF is a five-part prompt framework that helps you give Copilot clear instructions while controlling what information it uses. Danh Nolan RCCOF for your clinical email Element What to include R — Role Act as a professional medical communications assistant. C — Context Email an internal colleague about a securely shared, de-identified case summary. C — Constraints Exclude patient identifiers, diagnoses, clinical details, and confidential information. O — Output Brief, professional email with a subject, greeting, and closing. F — Few-shot Provide a generic example of the desired writing style. Copy this RCCOF prompt into Copilot Ready-to-use prompt Copy Role: Act as a professional medical communications assistant. Context: I am introducing a de-identified case

summary securely shared via OneDrive with an internal colleague. Constraints: Do not include patient identifiers, diagnoses, clinical details, or confidential information. Do not retrieve or summarize files or emails. Output: Draft a professional email under 80 words with a subject line. Few-shot example: “I've shared a document securely for your review and would appreciate your feedback.” Final privacy check Before selecting Generate, verify that the prompt contains no patient information. After generation, review the email for unexpected details and confirm the recipient and OneDrive permissions before sending. Key principle: RCCOF improves prompt structure, but it does not independently guarantee confidentiality. Your organization's approved Copilot environment and privacy policies still govern its use.

Guidance-level audit: In Exchange 2, the learner instructs Dr. Chen to inspect the recipient, link type, access panel, and inherited rights while keeping the final send action with her. In Exchange 1, the learner limits Copilot to a nonclinical administrative message and asks Dr. Chen to review the recipient, link, and draft. In Exchange 3, the learner specifies pre-generation input checks and post-generation inspection, without personally sharing or sending. Improvement needed: the original learner turns are long and contain copy/paste and example-email wording instead of concise one-action coaching with a screen-confirmation question. This audit reports rather than edits those actual turns.

Evidence-based coaching reflection (146 words):
My strongest coaching choice occurred in Exchange 2, when Dr. Chen asked how to confirm access before sending. I directed her to inspect the recipient, Specific people link, view permission, and Manage access rather than taking control of the sharing decision. Her subsequent reply confirmed that she had checked the new link and inherited access herself. This preserved clinician ownership. One improvement concerns Exchange 1: my lengthy response included a ready-made prompt and multiple instructions at once, and the simulation returned a message-length warning. I shortened the guidance afterward, but I should have started with a single action and asked what she saw on screen. In Exchange 3, I described post-generation review but did not observe the generated draft. The transcript ends with her request for a two-part verification plan, without my answer. Next time I will complete that verification step explicitly before closing the session.

Final pre-submission check

☑️ Synthetic content only; no real identifiers, secrets, or tenant data introduced.

☑️ No formal HIPAA de-identification determination claimed.

☑️ No anonymous links, administrative override, clinical interpretation or technician sending.

☑️ Systems and conditional dependency impacts grounded in the Fact Sheet only.

☑️ Four-element policy classification and dual source / KB verification included.

☑️ Guide word count 600; 400–600 target MET.

☑️ Three verbatim exchange pairs integrated from uploaded role-play. Final generated-output review not demonstrated in transcript: Dr. Chen’s final question has no learner response. Complete this in the role-play if assessment requires a post-generation verification exchange.

Transcript limitation and remaining role-play action

The final captured clinician question reads verbatim: That helps clarify how to structure the request. Before I hit generate, I want to make sure I know how to check if the output is actually safe to send. What should my two-part verification plan look like to ensure this draft meets both the factual accuracy of the message and our institutional policy standards?

No learner response to that question appears in the uploaded transcript. For maximal alignment with the 25-point rubric, complete the two-part verification conversation and capture a real post-generation source-fact and KB/policy review. Do not describe a hypothetical response as an actual exchange.